TakeSure — Demo Video Script

CodeSlayer 2.0 2K26 · Round 1 · Team Bug Hunters · Explaining the project idea only · Running time ≈ 2:35

Guidelines this script follows: 2–3 minutes · at least one face clearly visible (all three of you appear) · explains the idea only · no prototype, app screen, screen recording, link or QR code anywhere · everything spoken in future tense ("the user will…", "the system would…").

Timing plan

Time	Part	Speaker	On screen
0:00 – 0:15	Hook	Dhriti (on camera)	Dhriti holding a medicine strip and an ashwagandha bottle
0:15 – 0:25	Who we are & the idea in one line	Dhriti	Slide 1
0:25 – 0:50	The problem	Khushi (on camera, then slide)	Slide 2 stat tiles
0:50 – 1:45	How TakeSure would work	Shiwani (on camera, then slide)	Slide 3 flowchart, pointer moving down
1:45 – 2:10	What makes it different	Dhriti	Slide 3 novelty, then comparison table
2:10 – 2:35	Who it helps & close	Khushi, then all three	Slide 5 impact tiles, then all faces

About 350 spoken words at a calm 135–140 words per minute. If you end up under 2:30, pause a beat longer on each stat; if over 3:00, drop the Hindi lines first.

The script

0:00 – 0:15 · HOOK

Dhriti, full frame. Hold up the medicine strip, then the ashwagandha bottle.

"Meri mummy thyroid ki dawai leti hain — aur stress ke liye ashwagandha bhi."

"Nobody ever told her that together, these two can disturb her thyroid dose. In India, crores of people take Ayurveda and allopathy side by side — and there is no safety check between the two."

On screen: Dhriti on camera. Optional caption: "TakeSure · Confidence before taking something".

0:15 – 0:25 · WHO WE ARE

Dhriti, small in the corner over slide 1.

"We are Team Bug Hunters — Dhriti, Shiwani and Khushi — and our idea is **TakeSure**: a safety checker for Ayurvedic products taken alongside allopathic medicines. Two photos in, one sourced safety verdict out."

On screen: slide 1.

0:25 – 0:50 · THE PROBLEM

Khushi, full frame for the first sentence, then over slide 2.

"How big is this gap? In a hospital study, **37 percent** of patients were taking herbal and allopathic medicines together. Only about **19 percent** of Indian patients ever tell their doctor. And the Ministry of Ayush has received just about **1,473** adverse-event reports since 2018 — huge under-reporting."

"The doctor rarely asks, the patient rarely tells, the chemist has no tool, and the label says nothing. Tools like Medscape or 1mg only check allopathic medicines against each other. Nobody checks Thyronorm against ashwagandha."

On screen: slide 2; zoom on each number as it is said.

0:50 – 1:45 · HOW TAKESURE WOULD WORK

Shiwani, full frame for the first line, then over the slide-3 flowchart with a pointer moving top to bottom.

"Here is how TakeSure would work, in five steps."

"**One — Scan.** The user photographs the prescription and the Ayurvedic product label, or types the names. OCR reads them, and every name is shown back for confirmation before anything is checked."

"**Two — Match.** Indian brands become generics — Thyronorm becomes levothyroxine — and products become their herbs, so Chyawanprash maps to amla, ashwagandha and the rest."

"**Three — Decide.** A rule engine looks up every herb–medicine pair in a curated knowledge base, where each entry has a severity, an evidence grade — A, B or C — and a published source. The answer is Green, Amber, Red — or 'Not in database' when we don't know. We never guess."

"**Four — Explain.** Only now does AI come in: it rewrites the matched evidence in simple Hindi or English and gives three questions to ask the doctor."

"**Five — Report.** A one-page PDF for the doctor, and a suspected-reaction report pre-filled in the Ministry of Ayush format."

On screen: the flowchart only. No app screen. A hand-drawn paper sketch of the result card is fine if you want a visual.

1:45 – 2:10 · WHAT MAKES IT DIFFERENT

Dhriti over slide 3's novelty box, then the comparison table.

"Two things make TakeSure different. First: **the verdict comes from published data — the AI only explains.** The risk level never comes from a language model; it comes from a cited knowledge-base row. Switch the AI off, and the verdict is identical. This is not a ChatGPT wrapper."

"Second: **the bridge table nobody has built** — Indian brand names, Ayurvedic herb names in Sanskrit, Hindi and Latin, and graded evidence, in one open table. Western databases are English-only and don't know Himalaya or Dabur."

On screen: novelty bullets, then the star-rating table for about 6 seconds.

2:10 – 2:35 · WHO IT HELPS & CLOSE

Khushi over slide 5, then cut to all three on camera.

"For patients, it means knowing the risk before the first dose. For doctors and vaidyas, it means patients finally disclose what they take. For the Ministry of Ayush, it means structured reports and real data on what India is actually mixing."

All three, on camera: "We're not building another AI chatbot — we're building the missing safety layer between two medical systems. **TakeSure.** Lene se pehle, sure ho jao. Thank you."

On screen: all three faces; text "TakeSure · Team Bug Hunters · CodeSlayer 2K26". No link, no QR code.

Before you upload

- Record each part separately in landscape, 1080p, near a window, with earphone mic; 2–3 takes each.
- Edit in Clipchamp or CapCut: slide image full-screen, speaker in a corner; auto-captions on.
- Check: 2:30–3:00 long · faces visible · nothing on screen that looks like a working product · no "we built / it works" anywhere.
- Export MP4 → Google Drive → "Anyone with the link can view" → paste the link on slide 1 → test from another phone → re-export the PPT as PDF.